

MEDICATION REQUESTED: ☐

*The submission of this form does not guarantee approval and coverage of the medication.
 This form allows the pharmacy benefit manager to review the use of the medication for coverage. Documentation is required.*

PATIENT INFORMATION:

Date:	Client ID:	Call Log ID:
Patient Name:	DOB:	Patient ID:
Physician Name:	Specialty:	
Physician Phone:	Physician Fax:	

PHYSICIAN USE ONLY:

Dose:	Quantity:	Day Supply:	Expected Duration of Therapy:	Weight: _____ KG / LB (circle)	Height: _____ CM / IN (circle)
Directions:					

Diagnosis: ☐ Type 2 Diabetes (T2D) ☐ Cardiovascular Risk Reduction with T2D
☐ Risk Reduction of sustained eGFR decline with T2D and Chronic Kidney Disease (CKD)
☐ Other (specify): _____

List Diagnosis Codes: _____

All Initial Requests: Does the patient have a diagnosis of Type 2 Diabetes? ***Please provide copy of diagnostic A1c*** ☐ YES ☐ NO

Is the patient using the medication for weight loss? ☐ YES ☐ NO

If **YES**, is the prescriber willing to switch to an FDA-approved weight loss drug: SAXENDA, WEGOVY, or ZEPBOUND? ☐ YES ☐ NO

**If switching to one of the three drugs listed above, please circle the drug name and send a new prescription to the pharmacy.*

Approval is contingent on meeting clinical criteria as well as plan design and whether there is coverage for weight loss drugs.

Has the patient trialed metformin at maximum tolerated dose (up to 2550mg daily) or insulin for at least 90 days and failed to reach an A1c <7%? ***Chart notes documenting trial and corresponding A1c required.*** ☐ YES ☐ NO

Initial Requests for Cardiovascular Risk Reduction with Established Cardiovascular Disease and Type 2 Diabetes:

Has the patient previously had a stroke or MI, or does the patient have peripheral arterial disease? ☐ YES ☐ NO

Is the patient managed per current standard of care for established CVD, including lipid-lowering therapy, platelet aggregation inhibitors, ACE-inhibitors or ARBs, and/or beta blockers, as indicated? ***Documentation required.*** ☐ YES ☐ NO

Does the patient have a contraindication to or has the patient failed at least 90 days of treatment with an agent containing dapagliflozin (i.e., FARXIGA), canagliflozin (i.e., INVOKANA), or empagliflozin. (i.e., JARDIANCE)? ***Documentation required.*** ☐ YES ☐ NO

Initial Requests for Risk Reduction of sustained eGFR decline with T2D and Chronic Kidney Disease (CKD):

Does the patient have a diagnosis of chronic kidney disease (CKD) with sustained eGFR decline? ☐ YES ☐ NO

Please provide patient's eGFR: _____ mL/min ***Documentation required.***

Does the patient have a contraindication to or has the patient failed at least 90 days of treatment with an agent containing dapagliflozin (i.e., FARXIGA), canagliflozin (i.e., INVOKANA), or empagliflozin. (i.e., JARDIANCE)? ***Documentation required.*** ☐ YES ☐ NO

All Reauthorization Requests:

Has the patient's dose been optimized, increasing as tolerated to the maximum dose which achieves the patient's A1c goal? ☐ YES ☐ NO

Have chart notes and recent A1c labs within the past year been provided documenting follow-up and patient assessment for side effects e.g., bone density/muscle mass loss, gastrointestinal symptoms, and hypoglycemia? ☐ YES ☐ NO

For CV reauthorization: Has the patient had a major cardiovascular event (e.g., MI, stroke) since starting medication? ☐ YES ☐ NO

For CKD reauthorization: Has the patient had a reduction in expected decline of eGFR? Provide eGFR: _____ mL/min ☐ YES ☐ NO

****CHART NOTES DOCUMENTING DIAGNOSIS, PRIOR/CURRENT THERAPY, AND CORRESPONDING A1c LAB WORK REQUIRED.****

CURRENT THERAPY:

Medication(s)/Dose/Duration/Date

FAILED THERAPY:

Medication(s)/Dose/Duration/Date

ADDITIONAL NOTES: _____

Prescriber Signature: _____

Prescriber NPI: _____

Print Name: _____

Date: _____

The prescribing physician will need to complete this form and fax it, along with supporting documentation (charts/labs/etc.), back to BeneCard PBF at: Only a completed form will be considered for review.

This form is for the purpose of obtaining new or continued prescription treatment for the above member. Release of information via this form does not require member authorization. BeneCard PBF maintains strict adherence to the protection of members personally identifiable protected health information (PHI) under the HIPAA Act of 1996.